MediBuddy™

Your Trusted Buddy For Better Health

LAB REPORTS

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Dear MR THARANI GANESH MANOHARAN,

Greetings of the day!

Thank you for preferring Manipal Hospitals as your trusted healthcare partner.

Manipal Hospitals is the name synonym to clinical excellence, patient-centricity, and ethical practices. Our healthcare services are aligned towards providing you with personalized and comprehensive healthcare focusing prevention and overall wellness.

Our professional team of physicians, nurses, technicians, and support staff work with one goal to stay you healthy, always. To help you serve better, a helping hand from you is essential in meeting your healthcare requirements.

Here is how you can help,

• Please share all your current and past health issues with your physician as this history is crucial to plan your treatment. Do get your concerns clarified from our team. Stay connected with your doctor and the team.
• Strictly follow the treatment plan recommended by your physician. Your diet plays an important role in faster recovery, and overall wellbeing. Please follow the prescribed diet plan and always stay appropriately nourished through a balanced diet.
• Annual health check-ups are an important way to know about your health status. Please get it done if advised by your physician.
• It's been said that "Your critics are your best friends". Your feedback helps us improve on our services, please share it as and when you would like to.
• For any medication management related inquires, an experienced clinical pharmacologist is available.

Please find enclosed here your visit notes and reports (If applicable)

• Physician report (Doctors Prescription)
• Dieticians notes (if applicable)
• Laboratory reports (if applicable)
• Radiology reports(if applicable)
• Other diagnostic reports (as applicable)

Hospital director

MANIPAL HOSPITAL BANGALORE

Manipal Hospital Old Airport Road
#98, Old Airport Road, Kodihalli, Bengaluru 560 017, Karnataka
P 1800 102 3222

Manipal Health Enterprises Pvt. Ltd.
CIN: U85110KA2010PTC052540
Regd. Off. The Annexe, #98/2, Rustom Bagh, Off. HAL Airport Road, Bengaluru 560 017, Karnataka
P +91 80 4936 0300 E info@manipalhospitals.com www.manipalhospitals.com

PATIENT SAFETY & QUALITY OF CARE
NABH
ACCREDITED

MC-2045

Name	Mr K Manikantan	**URN**	MH011165003
Episode	H00000376927	**Ordered By**	HEALTH CHECK
Age	46Y 3M 24D	**Request Date**	12/03/2026 14:42
Sex	Male	**Patient Location**	HC SERVICE MHB

Haematology

Complete Blood Counts

AUTHORISED
12/03/2026 19:31

Specimen No: 2601355551-1; Collection Date: 12/03/2026 14:44; Receive Date: 12/03/2026 18:30
Specimen: WHOLE BLOOD;

Test	Result	Units	Ref. Range	Method
WBC Count (TC)	6.30	$10^3/\mu L$	4.00-10.00	Flow Cytometry
RBC Count	5.87 #	million/ cu.mm	4.50-5.50	Electrical Impedance
Haemoglobin	16.50	g/dL	13.00-17.00	SLS Hb Detection
Haematocrit [PCV]	48.40 #	%	38.00-48.00	Electrical Impedance
MCV	82.50 #	fL	83.00-101.00	Calculation
MCH	28.10	pg	27.00-32.00	Calculation
MCHC	34.10	g/dL	31.50-34.50	Calculation
RDW	12.70	%	11.60-14.00	Calculation
Platelet Count	232.00	$10^3/\mu L$	150.00-450.00	Flow Cytometry
Neutrophils	42.80	%	40.00-75.00	Flow Cytometry
Lymphocytes	43.30	%	20.00-45.00	Flow Cytometry
Monocytes	9.00	%	2.00-10.00	Flow Cytometry
Eosinophils	4.30	%	0.00-7.00	Flow Cytometry
Basophils	0.60	%	0.00-1.00	Flow Cytometry
Neutrophil Absolute	2.70	$10^3/\mu L$	2.00-7.00	Calculation
Lymphocyte Absolute	2.73	$10^3/\mu L$	1.00-3.00	Calculation
Monocyte Absolute	0.57	$10^3/\mu L$	0.20-1.00	Calculation
Eosinophil Absolute	0.27	$10^3/\mu L$	0.04-0.44	Calculation
Basophil Absolute	0.04	$10^3/\mu L$	0.02-0.10	Calculation
nRBC	0.00	/100 WBC		

----------END OF REPORT----------

Dr SHRINIVAS V , M.D Pathology
Consultant Hematopathologist
KMC : 19863

MC-2045

Name	Mr K Manikantan	**URN**	MH011165003
Episode	H00000376927	**Ordered By**	HEALTH CHECK
Age	46Y 3M 24D	**Request Date**	12/03/2026 14:42
Sex	Male	**Patient Location**	HC SERVICE MHB

ESR

AUTHORISED
12/03/2026 21:27

Specimen No: 2601355551-1; Collection Date: 12/03/2026 14:44; Receive Date: 12/03/2026 18:30
Specimen: WHOLE BLOOD;

Test	Result	Units	Ref. Range	Method
ESR	3.00	mm / 1st hour	0.00-10.00	Capillary Photometry
MCV	83.30	fL	83.00-101.00	Calculation
MCH	26.90 #	pg	27.00-32.00	Calculation
MCHC	32.30	g/dL	31.50-34.50	Calculation
RDW	13.40	%	11.60-14.00	Calculation
Platelet Count	288.00	$10^3/\mu L$	150.00-450.00	Flow Cytometry
IPF	7.90	%	0.70-9.10	Flow Cytometry
Neutrophils	59.00	%	40.00-75.00	Flow Cytometry
Lymphocytes	30.40	%	20.00-45.00	Flow Cytometry
Monocytes	7.30	%	2.00-10.00	Flow Cytometry
Eosinophils	2.70	%	0.00-7.00	Flow Cytometry
Basophils	0.60	%	0.00-1.00	Flow Cytometry
Neutrophil Absolute	4.19	$10^3/\mu L$	2.00-7.00	Calculation
Lymphocyte Absolute	2.16	$10^3/\mu L$	1.00-3.00	Calculation
Monocyte Absolute	0.52	$10^3/\mu L$	0.20-1.00	Calculation
Eosinophil Absolute	0.19	$10^3/\mu L$	0.04-0.44	Calculation
Basophil Absolute	0.04	$10^3/\mu L$	0.02-0.10	Calculation
nRBC	0.00	/100 WBC		

----------END OF REPORT----------

Dr SHRINIVAS V , M.D Pathology
Consultant Hematopathologist
KMC : 19863

Peripheral Smear Examination

AUTHORISED
12/03/2026 19:45

Specimen No: 2601355551-1; Collection Date: 12/03/2026 14:44; Receive Date: 12/03/2026 18:30
Specimen: WHOLE BLOOD;

MC-2045

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Episode	H00000376927	**Ordered By**	HEALTH CHECK
Age	46Y 3M 24D	**Request Date**	12/03/2026 14:42
Sex	Male	**Patient Location**	HC SERVICE MHB

Peripheral Smear Examination

AUTHORISED
12/03/2026 19:45

Peripheral Smear

RBCs: Normocytic normochromic erythrocytes.

WBCs: Leukocytes are normal.

PLATELETS: Platelets are adequate

----------END OF REPORT----------

Dr SHRINIVAS V , M.D Pathology
Consultant Hematopathologist
KMC : 19863

Peripheral Smear Examination

AUTHORISED
14/03/2026 10:50

Specimen No: 2601391296-1; Collection Date: 14/03/2026 09:32; Receive Date: 14/03/2026 10:04
Specimen: WHOLE BLOOD;

Peripheral Smear

RBCs: Normocytic normochromic erythrocytes

WBCs: Leukocytes are normal.

PLATELETS: Platelets are adequate.

----------END OF REPORT----------

Dr SHRINIVAS V , M.D Pathology
Consultant Hematopathologist
KMC : 19863

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MC-2045

Name	Mr Tharani Ganesh Manoharan	**URN**	MH015417183
Episode	H00000377393	**Ordered By**	HEALTH CHECK
Age	25Y 5M 14D	**Request Date**	14/03/2026 08:49
Sex	Male	**Patient Location**	HC SERVICE MHB

Biochemistry

BUN (Blood Urea Nitrogen)

AUTHORISED
14/03/2026 11:17

Specimen No: 2601391295-1; Collection Date & Time: 14/03/2026 09:32; Receive Date & Time: 14/03/2026 10:06
Specimen: SERUM;

Test	Result	Units	Ref. Range	Method
Urea Nitrogen	9.80	mg/dL	6.00-20.00	Urease GLDH

----------END OF REPORT----------

Dr ROOPA MURGOD , M.D Biochemistry
Consultant Biochemist
KMC : 44918

Calcium - Total

AUTHORISED
14/03/2026 11:19

Specimen No: 2601391295-1; Collection Date & Time: 14/03/2026 09:32; Receive Date & Time: 14/03/2026 10:06
Specimen: SERUM;

Test	Result	Units	Ref. Range	Method
CALCIUM TOTAL	10.25	mg/dL	8.60-10.30	NM-BAPTA

----------END OF REPORT----------

Dr ROOPA MURGOD , M.D Biochemistry
Consultant Biochemist
KMC : 44918

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MC-2045

Name	Mr Tharani Ganesh Manoharan	**URN**	MH015417183
Episode	H00000377393	**Ordered By**	HEALTH CHECK
Age	25Y 5M 14D	**Request Date**	14/03/2026 08:49
Sex	Male	**Patient Location**	HC SERVICE MHB

Creatinine

AUTHORISED
14/03/2026 11:19

Specimen No: 2601391295-1; Collection Date & Time: 14/03/2026 09:32; Receive Date & Time: 14/03/2026 10:06
Specimen: SERUM;

Test	Result	Units	Ref. Range	Method
CREATININE	0.98	mg/dL	0.80-1.60	Modified Jaffe's
eGFR	106.67	ml/min/1.73sq.m	>60.00	Calculation

----------END OF REPORT----------

Dr ROOPA MURGOD , M.D Biochemistry
Consultant Biochemist
KMC : 44918

GGT (Gamma Glutamyl Transpeptidase)

AUTHORISED
14/03/2026 11:18

Specimen No: 2601391295-1; Collection Date & Time: 14/03/2026 09:32; Receive Date & Time: 14/03/2026 10:06
Specimen: SERUM;

Test	Result	Units	Ref. Range	Method
GGT Gamma Glutamyl Transpeptidase	22.00	U/L	10.00-60.00	Enzymatic

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Dr ROOPA MURGOD , M.D Biochemistry
Consultant Biochemist
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Name	Mr Tharani Ganesh Manoharan	**URN**	MH015417183
Episode	H00000377393	**Ordered By**	HEALTH CHECK
Age	25Y 5M 14D	**Request Date**	14/03/2026 08:49
Sex	Male	**Patient Location**	HC SERVICE MHB

Glycated Hemoglobin (HbA1C)

AUTHORISED
14/03/2026 10:49

Specimen No: 2601391295-2; Collection Date & Time: 14/03/2026 09:32; Receive Date & Time: 14/03/2026 10:13
Specimen: WHOLE BLOOD;

Test	Result	Units	Ref. Range	Method
HbA1c (Glycosylated Hemoglobin)	5.40	%	<6.50	HPLC
Estimated Average Glucose (eAG)	108.28	mg/dL		

Reference ranges:
Nondiabetic adults: < 5.7 %
Prediabetes (At Risk): 5.7 % - 6.4 %
Diabetic Range: > 6.5 %

Use:
1.Monitoring compliance and long-term blood glucose level control in patients with diabetes
2.Index of diabetic control (direct relationship between poor control and development of complications).
3. Predicting development and progression of diabetic microvascular complications.

Limitations:
1. A1C values may be falsely elevated or decreased in those with chronic kidney disease.
2.False elevations may be due in part to analytical interference from carbamylated hemoglobin formed in the presence of elevated concentrations of urea, with some assays.
3. False decreases in measured A1C may occur with hemodialysis and altered red cell turnover, especially in the setting of erythropoietin treatment

References:
Rao.L.V.,Michael snyder.L.(2021).Wallach's Interpretation of Diagnostic Tests.
11th Edition. Wolterkluwer. NaderRifai,Andrea Rita Horvath,Carl T.wittwer.(2018)Teitz Text book of Clinical Chemistry and Molecular Diagnostics.First edition,Elsevier,South Asia.

----------END OF REPORT----------

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Episode	H00000377393	**Ordered By**	HEALTH CHECK
Age	25Y 5M 14D	**Request Date**	14/03/2026 08:49
Sex	Male	**Patient Location**	HC SERVICE MHB

Iron With TIBC

AUTHORISED
14/03/2026 11:19

Specimen No: 2601391295-1; Collection Date & Time: 14/03/2026 09:32; Receive Date & Time: 14/03/2026 10:06
Specimen: SERUM;

Test	Result	Units	Ref. Range	Method
IRON	76.00	µg/dl	33.00-193.00	Ferene
Total Iron Binding Capacity	330.00 #	µg/dl	90.00-193.00	Ferene
TRANSFERRIN SATURATION	23.03	%	20.00-50.00	Calculation
UIBC	254.00	µg/dl	155.00-355.00	Ferene

Interpretation of Iron status must be correlated with other parameters given below as a whole study rather than interpreting a single test.
1. Measurements of serum iron, TIBC and the percentage of iron saturation of transferring are useful screening tests for iron
2. However, serum iron exhibits significant diurnal variation and may transiently rise or reach reference values after dietary or
3. The diagnostic specificity of a low serum iron for iron deficiency is lost in the presence of acute & chronic inflammatory processes as the concentrations of iron and transferrin in the serum are significantly affected, and fall rapidly as part of the acute phase response irrespective of the iron stores status in the body.

----------END OF REPORT----------

Dr ROOPA MURGOD , M.D Biochemistry
Consultant Biochemist
KMC : 44918

Liver Function Test

AUTHORISED
14/03/2026 11:19

Specimen No: 2601391295-1; Collection Date & Time: 14/03/2026 09:32; Receive Date & Time: 14/03/2026 10:06
Specimen: SERUM;

Test	Result	Units	Ref. Range	Method
BILIRUBIN - TOTAL	0.79	mg/dL	0.30-1.20	DIAZO
BILIRUBIN - DIRECT	0.27 #	mg/dL	0.00-0.20	DIAZO
BILIRUBIN - INDIRECT	0.52	mg/dL	0.20-1.00	Calculation

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Name	Mr Tharani Ganesh Manoharan	**URN**	MH015417183
Episode	H00000377393	**Ordered By**	HEALTH CHECK
Age	25Y 5M 14D	**Request Date**	14/03/2026 08:49
Sex	Male	**Patient Location**	HC SERVICE MHB

Liver Function Test

AUTHORISED
14/03/2026 11:19

Test	Result	Units	Ref. Range	Method
SGOT/AST	15.00	U/L	10.00-40.00	UV With P5P
SGPT/ALT	18.00	U/L	10.00-40.00	UV With P5P
ALP	101.00	U/L	45.00-135.00	PNPP
TOTAL PROTEIN	7.7	g/dL	6.0-8.0	Biuret
ALBUMIN	4.8	g/dL	3.5-5.2	BCG
GLOBULIN	2.9	g/dL		Calculation
ALB/GLOB (A / G) Ratio	1.66			Calculation
GGT Gamma Glutamyl Transpeptidase	22.00	U/L	10.00-60.00	Enzymatic

Note:
*NEW BORN:Vary according to age (days), body wt & gestation of baby
*New born: 4 times the adult value

----------END OF REPORT----------

Dr ROOPA MURGOD , M.D Biochemistry
Consultant Biochemist
KMC : 44918

Thyroid Profile (T3 T4 TSH)

AUTHORISED
14/03/2026 11:15

Specimen No: 2601391295-1; Collection Date & Time: 14/03/2026 09:32; Receive Date & Time: 14/03/2026 10:06
Specimen: SERUM;

Test	Result	Units	Ref. Range	Method
T3 - Triiodothyronine	1.200	ng/mL	0.700-2.040	ECLIA
T4 - Thyroxine	8.37	µg/dl	4.50-11.00	ECLIA
Thyroid Stimulating Hormone	4.740 #	µIU/mL	0.340-4.250	ECLIA

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Name	Mr Tharani Ganesh Manoharan	**URN**	MH015417183
Episode	H00000377393	**Ordered By**	HEALTH CHECK
Age	25Y 5M 14D	**Request Date**	14/03/2026 08:49
Sex	Male	**Patient Location**	HC SERVICE MHB

Thyroid Profile (T3 T4 TSH)

AUTHORISED
14/03/2026 11:15

TSH levels are subject to circadian variation, reaching peak levels between 2-4.a.m.and at a minimum between 6-10 pm.Factors such as change of seasons hormonal fluctuations,Ca or Fe supplements,high fibre diet,stress and illness affect TSH results.

----------END OF REPORT----------

Dr ROOPA MURGOD , M.D Biochemistry
Consultant Biochemist
KMC : 44918

Transferrin Saturation

AUTHORISED
14/03/2026 11:19

Specimen No: 2601391295-1; Collection Date & Time: 14/03/2026 09:32; Receive Date & Time: 14/03/2026 10:06
Specimen: SERUM;

Test	Result	Units	Ref. Range	Method
Total Iron Binding Capacity	330.00 #	µg/dl	90.00-193.00	Ferene
TRANSFERRIN SATURATION	23.03	%	20.00-50.00	Calculation
IRON	76.00	µg/dl	33.00-193.00	Ferene

----------END OF REPORT----------

Dr ROOPA MURGOD , M.D Biochemistry
Consultant Biochemist
KMC : 44918

Uric Acid - Serum

AUTHORISED
14/03/2026 11:17

Specimen No: 2601391295-1; Collection Date & Time: 14/03/2026 09:32; Receive Date & Time: 14/03/2026 10:06
Specimen: SERUM;

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Name	Mr Tharani Ganesh Manoharan	**URN**	MH015417183
Episode	H00000377393	**Ordered By**	HEALTH CHECK
Age	25Y 5M 14D	**Request Date**	14/03/2026 08:49
Sex	Male	**Patient Location**	HC SERVICE MHB

Uric Acid - Serum

AUTHORISED
14/03/2026 11:17

Test	Result	Units	Ref. Range	Method
URIC ACID	6.60	mg/dL	3.50-7.20	Uricase

----------END OF REPORT----------

Dr ROOPA MURGOD , M.D Biochemistry
Consultant Biochemist
KMC : 44918

Vitamin B12

AUTHORISED
14/03/2026 11:08

Specimen No: 2601391295-1; Collection Date & Time: 14/03/2026 09:32; Receive Date & Time: 14/03/2026 10:06
Specimen: SERUM;

Test	Result	Units	Ref. Range	Method
VITAMIN B-12	279.00	pg/mL	250.00-1100.00	ECLIA

Reference Range:
Deficient: 32 - 246

Clinical Note:
Patients taking vitamin B12 supplementation may have misleading results Also recent vitamin B12 administration could result in normal or elevated serum concentrationsMany other conditions are known to cause an increase or decrease in the serum vitamin B12 concentration including:
Increased Serum B12:
Ingestion of vitamin C, estrogens, vitamin A. Hepatocellular injury, Myeloproliferative disorder, Uremia etc.
Decreased Serum B12:
Pregnancy, Aspirin, Anticonvulsants, Colchicine, Contraceptives, Smoking, Hemodialysis, Multiple myeloma, Ethanol ingestion etc.

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Episode	H00000377393	**Ordered By**	HEALTH CHECK
Age	25Y 5M 14D	**Request Date**	14/03/2026 08:49
Sex	Male	**Patient Location**	HC SERVICE MHB

Vitamin B12

AUTHORISED
14/03/2026 11:08

----------END OF REPORT----------

Dr ROOPA MURGOD , M.D Biochemistry
Consultant Biochemist
KMC : 44918

Total Vitamin D

AUTHORISED
14/03/2026 11:46

Specimen No: 2601391295-1; Collection Date & Time: 14/03/2026 09:32; Receive Date & Time: 14/03/2026 10:06
Specimen: SERUM;

Test	Result	Units	Ref. Range	Method
VITAMIN D (25 - Hydroxy)	11.70	ng/mL		ECLIA

Reference ranges:
Deficiency:< 20 ng/ml
Insufficiency: 20-29 ng/ml
Optimum level: 30-80 ng/ml
Possible toxicity :> 150 ng/mL

----------END OF REPORT----------

Dr ROOPA MURGOD , M.D Biochemistry
Consultant Biochemist
KMC : 44918

Lipid Profile

AUTHORISED
14/03/2026 11:19

Specimen No: 2601391295-1; Collection Date & Time: 14/03/2026 09:32; Receive Date & Time: 14/03/2026 10:06
Specimen: SERUM;

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MC-2045

Name	Mr Tharani Ganesh Manoharan	**URN**	MH015417183
Episode	H00000377393	**Ordered By**	HEALTH CHECK
Age	25Y 5M 14D	**Request Date**	14/03/2026 08:49
Sex	Male	**Patient Location**	HC SERVICE MHB

Lipid Profile

AUTHORISED
14/03/2026 11:19

Test	Result	Units	Ref. Range	Method
TOTAL CHOLESTEROL	139.00	mg/dL	<200.00	CHOD / POD
Reference Ranges: Desirable: <200 Moderate Risk:200-239 High risk: >240				
TRIGLYCERIDES	69.00	mg/dL	<150.00	GPO / POD
Reference ranges: Desirable: <150 mg/dL High: 200 - 499 mg/dL Very high: >500 mg/dL Borderline high:151-199 mg/dL				
HDL - CHOLESTEROL	36.50 #	mg/dL	40.00-60.00	Enzymatic
VLDL - CHOLESTEROL	13.80	mg/dL	2.00-30.00	Calculation
LDL - CHOLESTEROL	113.00 #	mg/dL	<100.00	Enzymatic
Reference ranges: Optimal:<100 mg/dL Near/Above optimal:100-129 mg/dL Borderline High:130-159 mg/dL High Risk:160-189 mg/dL				
T.CHOL/HDL.CHOL Ratio	3.81			Calculation
Reference ranges: Optimal - <4.0 Borderline - 4.0-5.0 High Risk - >6				
LDLCHOL / HDLCHOL Ratio	3.10			Calculation
Reference ranges: Optimal - <3 Borderline -3-4 High Risk - >6				
Non HDL Cholesterol	102.50			

LDL-Cholesterol treatment goals based on risk categories proposed by Lipid Association of India (LAI)

Risk Categories	**Treatment Goals**	
	LDL-C in mg/dL	**non HDL-C in mg/dL**

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F-401B R2

Name	Mr Tharani Ganesh Manoharan	**URN**	MH015417183
Episode	H00000377393	**Ordered By**	HEALTH CHECK
Age	25Y 5M 14D	**Request Date**	14/03/2026 08:49
Sex	Male	**Patient Location**	HC SERVICE MHB

Lipid Profile

AUTHORISED
14/03/2026 11:19

Extreme high risk - Category A (CAD with >=1 feature of high risk group)	<50 (Optional <30)	<80 (Optional <60)
Extreme high risk - Category B (CAD with >1 feature of high risk group or recurrent ACS LDL-C <=50 mg/dL)	<30	<60
Very High Risk (Pre-existing ASCVD, Diabetes with 2 others major ASCVD factors, Familial Homozygous Hypercholesterolemia)	<50	<80
High Risk (>=3 major ASCVD risk factors or any target organ damage or >2 other major ASCVD risk factors >=1 other high-risk features)	<70	<100
Moderate Risk (2 other major ASCVD risk factors. Low risk group 1 moderate risk factor. Lifetime CVD risk >=30%)	<100	<130
Low Risk (0 - 1 major ASCVD risk factors and Lifetime CVD risk <30%)	<100	<130

Reference:
Lipid Association of India Expert Consensus statement on Management of Dyslipidemia in Indians 2020: Part III

----------END OF REPORT----------

Dr ROOPA MURGOD , M.D Biochemistry
Consultant Biochemist
KMC : 44918

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LIFE'S ON

MC-2045

Name	Mr Tharani Ganesh Manoharan	**URN**	MH015417183
Episode	H00000377393	**Ordered By**	HEALTH CHECK
Age	25Y 5M 14D	**Request Date**	14/03/2026 08:49
Sex	Male	**Patient Location**	HC SERVICE MHB

Clinical Pathology

Urine Routine and Microscopy (Qualitative Method)

AUTHORISED
14/03/2026 16:31

Specimen No: 2601391297-1; Collection Date: 14/03/2026 09:31; Receive Date: 14/03/2026 14:40
Specimen: URINE;

Test	Result	Units	Ref. Range	Method
MACROSCOPIC DESCRIPTION				
Colour	Straw-yellow		Pale Yellow - Yellow	
Appearance	CLEAR			
Reaction[pH]	5.5		4.5-8.8	pH Indicator Method
Specific Gravity	1.014 #		1.015-1.025	Refractivity Method
CHEMICAL EXAMINATION				
Protein	Negative		NIL-TRACE	Protein Error of pH Indicator
Glucose	NOT DETECTED		NIL	Enzymatic
Ketone Bodies	NOT DETECTED			Nitroprusside Method
Urobilinogen	NIL		NORMAL	Azo Coupling Method
Bile Salts	NEGATIVE		NEGATIVE	
Bile Pigments	NEGATIVE		NIL	Azo Coupling Method
Nitrite	NEGATIVE		NEGATIVE	Griess Method
MICROSCOPIC EXAMINATION				
WBC / Pus Cells	0	/HPF	0-6	
Red Blood Cells	0	/HPF	0-2	
Epithelial Cells	0.10	/HPF	0-4	
Casts	NIL		NIL	
Crystals Urine	NIL		ABSENT	

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MC-2045

Name	Mr Tharani Ganesh Manoharan	**URN**	MH015417183
Episode	H00000377393	**Ordered By**	HEALTH CHECK
Age	25Y 5M 14D	**Request Date**	14/03/2026 08:49
Sex	Male	**Patient Location**	HC SERVICE MHB

Urine Routine and Microscopy (Qualitative Method)

AUTHORISED
14/03/2026 16:31

----------END OF REPORT----------

Dr SHRINIVAS V , M.D Pathology
Consultant Hematopathologist
KMC : 19863

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F-401B R2

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LIFE'S ON

NAME	Tharani GANESH MANOHARAN	STUDY DATE	14/03/2026 9:42AM
AGE / SEX	25 y / M	HOSPITAL NO.	MH015417183
ACCESSION NO.	R12729432	MODALITY	CR
REPORTED ON	14/03/2026 10:16AM	REFERRED BY	HEALTH CHECK

CHEST X-RAY PA

No cardiomegaly. Pulmonary vasculature is normal.

Lungs are clear.

Hila are not enlarged.

No mediastinal widening.

CP angles are clear.

Diaphragm is normal in position.

Mild scoliosis of the thoracic spine with convexity toward the right side.

Needs correlation with clinical findings and other investigations.

Dr. Sabina Toppo

CONSULTANT RADIOLOGIST

********End Of Report********

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LIFE'S ON

NAME	Tharani GANESH MANOHARAN	STUDY DATE	14/03/2026 12:07PM
AGE / SEX	25 y / M	HOSPITAL NO.	MH015417183
ACCESSION NO.	R12729431	MODALITY	US
REPORTED ON	14/03/2026 3:33PM	REFERRED BY	HEALTH CHECK

Sp/pk

REAL TIME ULTRASONOGRAPHY OF THE ABDOMEN AND PELVIS WAS PERFORMED

LIVER: Span measures 12.6 cm. Normal in size, shape and outline. **Grade I fatty changes noted.** No focal cystic or solid mass lesion. No intrahepatic biliary dilatation. Intrahepatic portion of IVC is normal. Portal vein is normal. Portahepatis is normal.

GALL BLADDER: **Partially distended. No calculi.**

C.B.D: Not dilated.

PANCREAS: Parenchymal texture normal. No ductal dilatation. No calculi.

SPLEEN: Measures 10.6 cm. Normal texture.

KIDNEYS: Normal in size, shape and outline. Parenchymal texture normal. Corticomedullary differentiation is well made out and is normal. Sinus echoes are normal.
Right kidney measures 10.5 cm with a parenchymal thickness of 1.4 cm.
Left kidney measures 11 cm with a parenchymal thickness of 1.7 cm.

URINARY BLADDER: Wall thickness normal. Luminal echoes normal. No calculi.

PROSTATE: Volume 14 cc, not enlarged.

No free fluid in peritoneal cavity.

IMPRESSION:

Grade I fatty liver.

Dr. KOTA ASHWIN REDDY

CONSULTANT RADIOLOGIST

********End Of Report********

Mr Tharani G
ID: 15417183

Vital Signs™ 226 166 05

14.03.2026 8:52:44
MANIPAL HEALTH CHECK
OLD AIRPORT ROAD
BANGALORE

25 Years

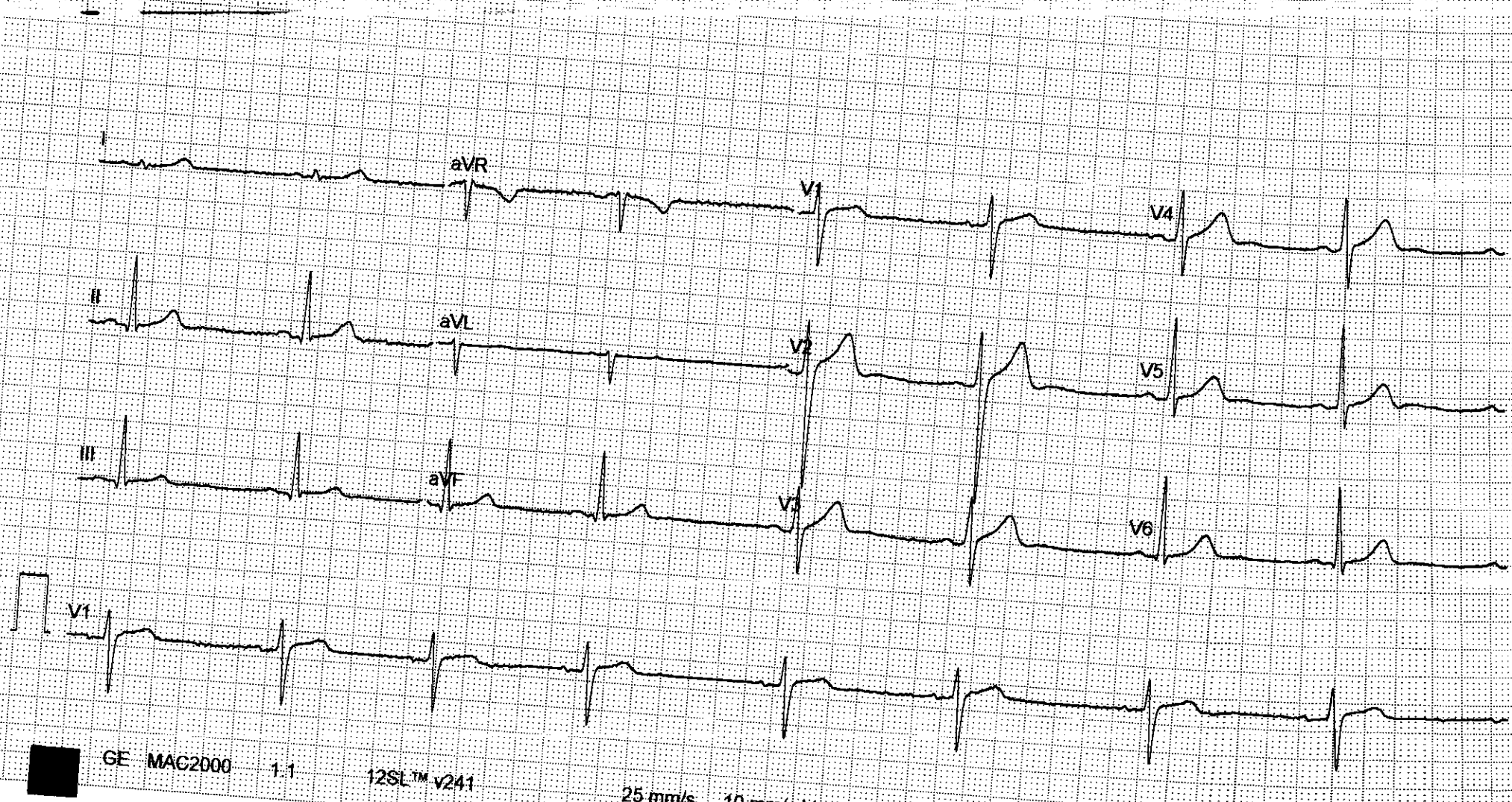
Male

CE 0537 LOT D 855

49 bpm

-- / -- mmHg

QRS	94 ms
QT / QTcBaz	430 / 388 ms
PR	166 ms
P	112 ms
RR / PP	1224 / 1224 ms
P / QRS / T	56 / 87 / 58 degrees



GE MAC2000 1.1 12SL™ v241 25 mm/s 10 mm/mV ADS 0.56-20 Hz 50 Hz Unconfirmed

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